

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think>

Finding Reasoning Paths :

1 . **Impulsivity , poor attention , short attention span , hyperactivity** â‡™ Attention - Deficit /Hyperactivity Disorder (ADHD) â‡™ Behavioral therapy or stimulant medication .

2 . **Impulsivity , hyperactivity , poor concentration , forgetfulness** â‡™ ADHD â‡™ Behavioral therapy or stimulant medication .

3 . **Frequent self-harm** â‡™ Excoriation disorder â‡™ Behavioral therapy or treatment for self-harming behavior .

4 . **Major depressive disorder in family history** â‡™ Mental health disorder â‡™ Consider ADHD or other psychiatric conditions .

5 . **Hyperactivity , impulsivity , forgetfulness , behavioral issues** â‡™ Neurodevelopmental disorder â‡™ ADHD â‡™ Behavioral therapy or stimulant medication .

6 . **Acute otitis media** â‡™ Potential for increased intracranial pressure â‡™ Exclude as a primary cause of symptoms .

7 . **Parental history of Graves ' disease** â‡™ No direct link to patient's symptoms .

Reasoning Process :

1 . **Core Symptoms Analysis** :

- The child exhibits impulsivity , poor attention , short attention span , hyperactivity , forgetfulness , and behavioral issues (e.g ., interrupting others , interrupting classes). These are hallmark features of **Attention - Deficit /Hyperactivity Disorder (ADHD)** .

- The symptoms are consistent with the **combined subtype** of ADHD , as they include both hyperactivity - impulsivity and inattention .

2 . **Exclusion of Alternatives** :

- The child does not have a history of self-harm (e.g ., cutting or breaking limbs). Therefore , diagnosing **excoriation disorder** is unlikely .

- The child does not have a history of other psychiatric disorders , such as autism spectrum disorder , oppositional defiant disorder , or conduct disorder .

- The child does not have a history of neurological or genetic conditions (e.g ., hearing impairment , neurofibromatosis) that could explain his symptoms .

- The child was born at term with no developmental delays , ruling out syndromic causes .

3 . **Symptoms and Development** :

- The symptoms began during early childhood and have persisted into adulthood , which aligns with the developmental course of ADHD .

- There is no evidence of other symptoms (e.g ., anxiety , mood swings , or mood disorders) that would suggest a different diagnosis .

4 . **Treatment Consideration** :

- ADHD is most commonly treated with **behavioral therapy** (e.g ., parent-training programs) and **stimulant medications** (e.g ., methylphenidate , amphetamines , or dextroamphetamine).

- Behavioral therapy addresses the core issues of impulsivity , hyperactivity , and inattention by teaching coping strategies , goal-setting , and social skills .

- Medications (e.g ., methylphenidate) are often used as an adjunct to behavioral therapy , especially if the symptoms are severe or if non-medical treatment fails .

Conclusion :

The most appropriate next step in treatment is **behavioral therapy** for this child , as it is the primary modality for managing ADHD and improving his ability to listen and concentrate .

</think>

<answer>

Behavior therapy

</answer>